NURSING INTERVIEW QUESTION BANK

21 Nursing Roles • Fresher, Junior & Senior Levels

Roles	Experience levels	Questions with answers
21	3 per role	315 total

How to use this bank: Select the nursing role and the candidate's experience level. Use the five questions as an interview set, or select individual questions to assess clinical knowledge, prioritization, patient safety and communication.

Clinical-use note: *These are model interview answers for preparation and assessment design. They are not clinical orders or substitutes for current guidelines, institutional protocols, local law, professional registration requirements or the candidate's authorized scope of practice. Have the question bank reviewed by an experienced registered nurse or nursing educator before recruitment use.*

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1. Staff Nurse

Fresher Level

Q1. What are the main responsibilities of a staff nurse?

Answer: A staff nurse assesses patients, plans and provides nursing care, administers prescribed medicines, monitors clinical changes, maintains records and communicates with the healthcare team. The nurse also supports hygiene, nutrition, mobility, patient education and safe discharge.

Q2. What are the five rights of medication administration?

Answer: The commonly taught rights are the right patient, medication, dose, route and time. Safe practice also includes checking allergies, indication, expiry, documentation and the patient's response, and clarifying any incomplete or unsafe order before administration.

Q3. How do you measure vital signs?

Answer: I identify the patient, explain the procedure and use suitable equipment to measure temperature, pulse, respiratory rate, blood pressure and oxygen saturation when indicated. I compare results with the patient's baseline and report abnormal values or concerning symptoms promptly.

Q4. What is infection control in nursing?

Answer: Infection control means preventing transmission of microorganisms through hand hygiene, appropriate PPE, aseptic technique, safe injection practices, environmental cleaning and correct waste disposal. Standard precautions apply to every patient, with additional precautions when indicated.

Q5. What is the nursing process?

Answer: The nursing process is a systematic approach: assessment, nursing diagnosis, planning, implementation and evaluation. It helps nurses identify patient needs, set measurable goals, provide individualized interventions and review whether care is effective.

Junior Level

Q1. A patient's vital signs suddenly become abnormal. What will you do?

Answer: I would assess the patient immediately rather than relying on the monitor alone. I would repeat measurements as appropriate, use an ABCDE approach, call for clinical assistance according to the escalation policy, initiate interventions within my competency and document the findings and response.

Q2. How do you prevent medication errors during a busy shift?

Answer: I use patient identifiers, check the prescription and allergies, calculate doses carefully, avoid interruptions where possible and follow independent double-check requirements for high-alert medicines. I document administration promptly and report near misses and errors transparently.

Q3. How would you manage a patient at risk of falling?

Answer: I assess fall risk, explain safety measures, keep the call bell and essential items within reach, ensure suitable footwear and adequate lighting, assist with transfers and follow the individualized care plan. I reassess after changes in condition or medication.

Q4. What would you do if a patient refuses treatment?

Answer: I would listen to the patient's concerns, assess understanding and decision-making capacity within my role, explain the purpose and likely consequences in clear language and notify the responsible clinician. I would respect applicable patient rights and document the discussion and refusal.

Q5. How do you give an effective shift handover?

Answer: I use a structured format such as SBAR and communicate identity, diagnosis, current condition, allergies, risks, recent changes, treatment, pending tests and priorities for the next shift. I invite questions and confirm that critical information has been understood.

Senior Level

Q1. How would you lead a ward during a sudden clinical deterioration involving multiple patients?

Answer: I would rapidly triage acuity, activate the emergency response for the most unstable patient and allocate competent staff to essential tasks. I would maintain clear communication, ensure escalation to medical leadership, redistribute workload and review each patient's status until the situation stabilizes.

Q2. How would you improve medication safety in your unit?

Answer: I would review incident and near-miss data, identify high-risk processes, standardize medication reconciliation and storage, reinforce independent checks for high-alert drugs and audit compliance. I would promote a non-punitive reporting culture while ensuring accountability and corrective action.

Q3. How do you manage repeated nursing documentation gaps?

Answer: I would determine whether the causes are workload, unclear standards, training gaps or system design. I would set clear documentation expectations, provide focused training, audit records, give timely feedback and monitor improvement rather than relying only on warnings.

Q4. How would you resolve conflict between two members of the nursing team?

Answer: I would address immediate patient-safety concerns first, then speak with the individuals objectively and privately. I would clarify facts, expectations and professional responsibilities, agree on a resolution and follow up. Persistent or serious conduct concerns would be escalated through policy.

Q5. Which indicators would you use to evaluate nursing quality?

Answer: I would monitor indicators such as falls, pressure injuries, medication incidents, healthcare-associated infections, missed nursing care, patient experience and documentation compliance. I would interpret trends alongside case mix and staffing, then implement and evaluate targeted improvements.

2. General Duty Staff Nurse

Fresher Level

Q1. What is the role of a general duty staff nurse?

Answer: A general duty staff nurse provides routine bedside care across general clinical areas. Duties include patient assessment, vital-sign monitoring, medication administration, hygiene, mobility assistance, documentation and reporting changes to the clinical team.

Q2. How do you identify a patient correctly?

Answer: I use at least two approved identifiers, such as the patient's full name and hospital identification number, and compare them with the wristband and clinical record. I do not use bed or room number as an identifier.

Q3. Why is hand hygiene important?

Answer: Hand hygiene reduces the transfer of microorganisms between patients, staff and the environment. I follow the recommended moments for hand hygiene and use soap and water or an alcohol-based hand rub according to the situation and infection-control policy.

Q4. What should be checked before giving an injection?

Answer: I verify the patient, medicine, dose, route, time, indication, allergies and expiry date. I use aseptic technique, select an appropriate site and equipment, explain the procedure and document the administration and response.

Q5. What is the purpose of a nursing record?

Answer: A nursing record provides a clear, timely account of assessment, care delivered, patient response and communication. It supports continuity, clinical decision-making, accountability and legal documentation.

Junior Level

Q1. How would you prioritize care for several patients?

Answer: I first identify patients with airway, breathing, circulation, neurological or other immediate safety concerns. I then prioritize time-critical medicines and procedures, reassess stable patients and delegate appropriate tasks while retaining responsibility for follow-up.

Q2. What would you do if a patient develops a pressure injury?

Answer: I would assess and document the site and skin findings, relieve pressure, inform the responsible clinician and follow the wound-care plan. I would reassess risk factors, repositioning, support surfaces, nutrition and moisture management to prevent progression.

Q3. How do you manage a patient with fever?

Answer: I assess temperature and associated symptoms, check other vital signs and look for signs of deterioration or infection. I inform the clinician as indicated, collect specimens only as ordered, administer prescribed treatment and monitor the response.

Q4. What would you do if a medication dose appears unusually high?

Answer: I would pause administration and verify the order, calculation, concentration, patient factors and relevant protocol with the prescriber or pharmacist. I would not guess or independently alter the prescription, and I would document the clarification.

Q5. How do you support a patient who needs help with daily activities?

Answer: I assess the patient's abilities and preferences, preserve privacy and encourage independence where safe. I assist with bathing, toileting, feeding and mobility as needed, use appropriate equipment and observe for pain, fatigue or changes in condition.

Senior Level

Q1. How would you maintain safe staffing on a general duty roster?

Answer: I would match staffing to patient acuity, workload, skill mix and required competencies, not simply patient numbers. I would identify coverage gaps early, escalate unsafe staffing, use approved contingency arrangements and monitor fatigue and continuity of care.

Q2. How would you respond to a serious patient-safety incident?

Answer: I would first protect the patient and arrange immediate clinical care. I would notify the appropriate leaders, preserve relevant records and equipment, document objective facts and support the formal review. Learning and system-level corrective actions should follow.

Q3. How would you improve compliance with infection-prevention practices?

Answer: I would assess baseline compliance, identify barriers, ensure supplies and training are available, conduct direct observations and provide feedback. I would review infection trends and work with infection-control personnel on measurable improvement actions.

Q4. How would you coach a nurse whose clinical performance is below standard?

Answer: I would identify specific observed gaps, assess knowledge and competency, agree on measurable improvement goals and provide supervised practice or education. I would document the plan, reassess performance and escalate if patient safety remains at risk.

Q5. How would you ensure continuity of care during patient transfers?

Answer: I would confirm transfer readiness, communicate current status, diagnosis, allergies, medication, lines, drains, risks and pending care using a structured handover. I would verify receiving-team acceptance and ensure records and essential equipment accompany the patient.

3. OT Nurse / Scrub Nurse

Fresher Level

Q1. What is the role of an OT nurse?

Answer: An operating-theatre nurse supports safe surgical care by preparing the theatre, checking equipment, maintaining asepsis, assisting the surgical team, accounting for instruments and supporting patient transfer and recovery.

Q2. What is the difference between a scrub nurse and a circulating nurse?

Answer: The scrub nurse works within the sterile field and handles sterile instruments and supplies. The circulating nurse works outside the sterile field, coordinates the room, obtains additional supplies, documents events and supports patient safety.

Q3. What is a sterile field?

Answer: A sterile field is an area prepared with sterile drapes, instruments and supplies to reduce contamination during a procedure. Only sterile items and appropriately scrubbed personnel may enter or handle the field.

Q4. What is the purpose of surgical hand antisepsis?

Answer: Surgical hand antisepsis reduces transient microorganisms and suppresses resident skin flora before sterile gowning and gloving. It must follow the approved product instructions and institutional procedure.

Q5. Why is a surgical count performed?

Answer: Counts of sponges, sharps and instruments help prevent retained surgical items. Counts are performed at specified stages by the designated team members and discrepancies are escalated and resolved before the procedure is concluded whenever possible.

Junior Level

Q1. What would you do if the sterile field is contaminated?

Answer: I would announce the contamination immediately, prevent use of affected items and replace contaminated instruments, drapes or gloves as required. I would not conceal the event; I would follow the team's corrective procedure and document or report it according to policy.

Q2. How do you prepare an operating theatre before a procedure?

Answer: I verify the schedule, room cleanliness, equipment function, required instruments, implants and consumables. I check patient-specific needs, availability of emergency equipment and the surgical safety checklist, then confirm readiness with the team.

Q3. What is the surgical time-out?

Answer: Time-out is a final team pause immediately before incision to confirm the correct patient, procedure, site and side, positioning, required imaging, antibiotics when relevant and anticipated critical events. Every team member should actively participate.

Q4. How would you handle a discrepancy in the instrument count?

Answer: I would inform the surgeon and circulating nurse immediately, repeat the count according to policy, search the sterile field and room and follow required imaging or escalation steps. The discrepancy must be resolved and documented before the patient leaves the relevant area, subject to emergency circumstances and policy.

Q5. How do you maintain specimen safety?

Answer: I verify specimen identity, source and required tests with the team, use the correct container and preservative, label at the point of care and ensure accurate handover. I clarify any mismatch before dispatch.

Senior Level

Q1. How would you manage an intraoperative emergency?

Answer: I would alert the team, clarify roles and support the immediate response while maintaining sterility where possible. I would ensure emergency equipment and supplies are available, communicate clearly, document key events and support transfer to the appropriate level of care.

Q2. How would you reduce surgical-site infection risk in the OT?

Answer: I would reinforce surgical hand antisepsis, appropriate skin preparation, sterile technique, traffic control, instrument sterilization and timely antimicrobial prophylaxis as prescribed. I would audit practices and investigate infection trends with the infection-control team.

Q3. How would you lead a surgical count process?

Answer: I would ensure standardized counts at required stages, verbal confirmation by designated staff and immediate escalation of discrepancies. I would avoid shortcuts during emergencies, ensure final reconciliation and review count-related incidents for system improvements.

Q4. How would you manage equipment failure during surgery?

Answer: I would inform the surgeon and team immediately, switch to a verified backup where available and coordinate technical support without compromising patient safety. I would quarantine faulty equipment, document the incident and ensure it is not returned to service until cleared.

Q5. How would you assess competency of OT nursing staff?

Answer: I would use structured competency checklists covering aseptic practice, instrument knowledge, counts, equipment, specimen handling, surgical safety and emergency response. I would combine observation, simulation and audit results, then plan supervised development.

4. ICU Trained Staff Nurse

Fresher Level

Q1. What is an ICU?

Answer: An Intensive Care Unit provides continuous monitoring and advanced treatment for patients with life-threatening illness or organ dysfunction. Care commonly involves ventilatory support, hemodynamic monitoring and coordinated multidisciplinary treatment.

Q2. What vital parameters are monitored in ICU?

Answer: Monitoring commonly includes heart rate, blood pressure, respiratory rate, oxygen saturation, temperature, consciousness, urine output and relevant laboratory results. The frequency and targets depend on the patient's condition and prescribed plan.

Q3. What is mechanical ventilation?

Answer: Mechanical ventilation is respiratory support delivered by a machine through an endotracheal tube or tracheostomy. It supports oxygenation and/or removal of carbon dioxide when a patient cannot maintain adequate breathing independently.

Q4. What is the purpose of a central venous catheter?

Answer: A central venous catheter provides access for selected medications, fluids, blood sampling and hemodynamic monitoring when indicated. It requires strict aseptic care and surveillance for infection, thrombosis and mechanical complications.

Q5. Why is intake and output monitoring important in ICU?

Answer: Accurate fluid balance helps assess circulation, kidney function and response to treatment. Nurses record all measurable intake and output, monitor trends and report unexpected changes promptly.

Junior Level

Q1. What would you do if a ventilator alarm suddenly sounds?

Answer: I would assess the patient first—colour, chest movement, oxygenation and clinical stability—then check the ventilator circuit and alarm cause. I would call for help, use the unit's emergency procedure and provide appropriate manual ventilation only if indicated and within competency.

Q2. How do you prevent ventilator-associated pneumonia?

Answer: I follow the current ventilator-care bundle, including hand hygiene, appropriate head-of-bed elevation when not contraindicated, oral care, assessment of sedation and readiness to wean, and safe suction practices. I also maintain circuit care according to policy.

Q3. What would you do if urine output suddenly decreases?

Answer: I would verify the measurement and check the catheter and drainage system for kinks or obstruction. I would assess blood pressure, perfusion and fluid balance, review recent trends and promptly inform the medical team because oliguria may indicate deterioration.

Q4. How do you monitor a patient receiving vasopressors?

Answer: I use the prescribed monitoring method and frequency, observe blood pressure and perfusion targets, assess the infusion site and pump settings, and monitor for arrhythmias or ischemia. I follow the unit's high-alert medication checks and never independently change the dose outside an authorized protocol.

Q5. What are signs of sepsis in an ICU patient?

Answer: Possible signs include altered temperature, tachycardia, tachypnea, hypotension, altered mental status, reduced urine output and worsening organ function. I recognize trends early, escalate promptly and support timely investigations and prescribed treatment.

Senior Level

Q1. How would you manage a patient with septic shock?

Answer: I would activate the sepsis response, assess ABCDE and coordinate rapid medical review. I would support timely cultures and investigations without delaying prescribed antimicrobials, fluid resuscitation and vasopressors when indicated, while reassessing perfusion, lactate, urine output and response.

Q2. How would you manage acute deterioration in a ventilated patient?

Answer: I would assess the patient before the ventilator, call for assistance and evaluate airway patency, tube position, oxygenation, circulation and ventilator/circuit function. I would coordinate immediate intervention with the intensivist and respiratory team and document the sequence and response.

Q3. How do you prevent ICU-acquired complications?

Answer: I would implement evidence-based bundles for infection prevention, pressure injury, delirium, thrombosis and device-related harm. I would review sedation, mobility, nutrition, skin and line necessity daily with the multidisciplinary team and audit adherence.

Q4. How would you lead an ICU team during a cardiac arrest?

Answer: I would activate the code response, assign clear roles, ensure high-quality CPR and defibrillator readiness, and use closed-loop communication. I would track medications and event times, support post-resuscitation stabilization and conduct a structured debrief.

Q5. How would you evaluate ICU nursing quality?

Answer: I would monitor risk-adjusted outcomes and process measures such as device-associated infections, pressure injuries, unplanned extubations, medication incidents, handover quality and bundle compliance. I would use audits and incident reviews to drive measurable improvements.

5. NICU Nurse

Fresher Level

Q1. What is NICU?

Answer: The Neonatal Intensive Care Unit provides specialized care for premature babies, low-birth-weight newborns and newborns with serious medical conditions. It provides continuous monitoring, respiratory support and specialized feeding.

Q2. What is the normal temperature range of a newborn?

Answer: The normal axillary temperature of a newborn is approximately 36.5–37.5°C. Maintaining this range is important because newborns are vulnerable to hypothermia and temperature instability.

Q3. What is neonatal jaundice?

Answer: Neonatal jaundice is yellow discoloration of a newborn's skin and eyes caused by increased bilirubin. Severe untreated hyperbilirubinemia can cause neurological injury, so assessment and timely treatment are important.

Q4. What is the purpose of an incubator?

Answer: An incubator provides a controlled environment for newborns, particularly premature infants. It helps maintain body temperature and provides appropriate humidity and protection.

Q5. What parameters are monitored in NICU?

Answer: Important parameters include temperature, heart rate, respiratory rate, oxygen saturation, blood glucose, feeding tolerance, weight and urine output. These help identify changes in the newborn's condition.

Junior Level

Q1. What would you do if a newborn develops respiratory distress?

Answer: I would assess breathing, respiratory rate, chest movements, colour and oxygen saturation. I would immediately inform the neonatologist, maintain airway positioning and assist with prescribed respiratory support. Continuous monitoring is essential.

Q2. How do you prevent hypothermia in premature babies?

Answer: I maintain the recommended incubator or radiant-warmer settings, minimize unnecessary exposure, use appropriate thermal protection and monitor temperature regularly. I follow skin-to-skin care recommendations when clinically appropriate.

Q3. How do you identify neonatal sepsis?

Answer: Possible signs include temperature instability, poor feeding, lethargy, respiratory distress, abnormal heart rate and changes in perfusion. I would immediately report these findings and assist with investigations and prescribed treatment.

Q4. What precautions are required during neonatal feeding?

Answer: I verify the prescribed feeding volume and method, assess feeding readiness and monitor tolerance. I follow infection-control procedures and observe for vomiting, abdominal distension, choking or aspiration signs.

Q5. What would you do if a premature baby develops apnea?

Answer: I would assess breathing and heart rate, stimulate the baby gently if indicated, ensure airway positioning and seek immediate assistance. I would provide respiratory support or resuscitation according to neonatal protocols and my competency.

Senior Level

Q1. How would you manage a newborn with suspected necrotizing enterocolitis (NEC)?

Answer: I would immediately escalate the concern to the neonatologist, monitor vital signs and abdominal findings, and follow orders regarding stopping feeds and gastric decompression. I would monitor fluid balance, blood glucose and signs of deterioration while assisting with investigations and treatment.

Q2. How would you manage a critically ill premature baby requiring respiratory support?

Answer: I would coordinate care with the neonatologist and respiratory team, monitor respiratory status and oxygenation, assess airway security and equipment function. I would also monitor temperature, circulation, feeding, fluid balance and potential complications.

Q3. What would you do if a newborn develops severe hypoglycemia?

Answer: I would recognize symptoms such as jitteriness, poor feeding, lethargy, apnea or seizures, confirm glucose according to protocol and urgently inform the neonatologist. I would assist with prescribed glucose treatment and repeat monitoring to evaluate response.

Q4. How would you supervise nurses working in NICU?

Answer: I would assess neonatal-care competency, supervise high-risk procedures and ensure adherence to hand hygiene, feeding safety and medication-dosing protocols. I would conduct regular training and review patient outcomes and documentation.

Q5. How would you communicate with parents of a critically ill newborn?

Answer: I would provide compassionate, clear and factual communication within my professional responsibilities. I would encourage questions, protect confidentiality and coordinate updates with the neonatologist. I would also support appropriate parental involvement in newborn care.

6. PICU Nurse

Fresher Level

Q1. What is PICU?

Answer: The Pediatric Intensive Care Unit provides specialized care for critically ill infants, children and adolescents. It provides continuous monitoring and advanced treatment for severe medical and surgical conditions.

Q2. What is the difference between NICU and PICU?

Answer: NICU primarily provides care for newborn babies, including premature infants. PICU provides critical care for children beyond the neonatal period, although age limits depend on hospital policy.

Q3. What are signs of respiratory distress in children?

Answer: Common signs include rapid breathing, chest retractions, nasal flaring, grunting, cyanosis and reduced oxygen saturation. These findings require prompt clinical assessment.

Q4. Why is weight important in pediatric medication administration?

Answer: Many pediatric medicines are prescribed according to body weight, usually in milligrams per kilogram. Accurate weight measurement is essential to prevent underdosing and medication toxicity.

Q5. What is pediatric fluid-balance monitoring?

Answer: It involves recording fluid intake and output to assess hydration and kidney function. Accurate monitoring is important because children can develop dehydration or fluid overload quickly.

Junior Level

Q1. How would you assess a child with breathing difficulty?

Answer: I would assess respiratory rate, work of breathing, oxygen saturation, colour and level of consciousness. I would identify warning signs such as retractions and grunting, alert the medical team and assist with prescribed respiratory support.

Q2. What would you do if a child develops a febrile seizure?

Answer: I would protect the child from injury, place them safely, time the seizure and maintain airway safety. I would not restrain the child or put anything in the mouth. I would seek medical assistance and monitor recovery.

Q3. How do you prevent medication errors in pediatric patients?

Answer: I verify the patient's weight, prescribed dose, concentration, route and administration rate. I perform required dose calculations and independent double-checks for high-risk medications according to hospital policy.

Q4. What are signs of dehydration in children?

Answer: Signs include dry mouth, reduced urine output, sunken eyes, reduced skin turgor, lethargy and tachycardia. Severe dehydration may cause poor perfusion and altered consciousness.

Q5. How do you communicate with parents of a child admitted to PICU?

Answer: I explain nursing procedures in simple language, listen to parental concerns and provide appropriate updates. I maintain confidentiality and involve the treating physician for detailed discussions about diagnosis and prognosis.

Senior Level

Q1. How would you manage a child with suspected septic shock?

Answer: I would activate emergency support, assess ABCDE and support airway and breathing. I would coordinate rapid vascular access, investigations, prescribed antimicrobial therapy and fluid or vasoactive support with the medical team. I would continuously reassess perfusion, mental status and urine output.

Q2. How would you manage a child with status epilepticus?

Answer: I would immediately activate emergency support, maintain airway safety, provide oxygenation and monitoring, and ensure appropriate access. I would assist with prescribed first-line anticonvulsant treatment and further escalation according to the pediatric emergency protocol.

Q3. How do you manage fluid overload in a critically ill child?

Answer: I assess respiratory status, edema, weight changes, fluid balance, urine output and relevant laboratory results. I immediately report concerning findings and coordinate prescribed fluid adjustments, diuretics or other treatment with the pediatric intensivist.

Q4. How would you lead a pediatric resuscitation team?

Answer: I would ensure clear role allocation, closed-loop communication and timely availability of emergency equipment. I would support protocol-based interventions, document events and medication administration, and participate in the post-resuscitation review.

Q5. How would you ensure pediatric medication safety across your department?

Answer: I would implement standardized weight-based dosing procedures, medication-concentration checks, independent verification for high-alert drugs and competency assessments. I would review medication incidents and improve systems that contribute to dosing errors.

7. Geriatric Nurse

Fresher Level

Q1. What is geriatric nursing?

Answer: Geriatric nursing focuses on the healthcare needs of older adults. It includes managing chronic diseases, supporting mobility, preventing complications and maintaining quality of life.

Q2. What are common health problems in elderly patients?

Answer: Common conditions include hypertension, diabetes, arthritis, osteoporosis, dementia, cardiovascular diseases and chronic kidney disease. Older adults may also experience sensory impairment and reduced mobility.

Q3. What is dementia?

Answer: Dementia is a syndrome involving deterioration in memory, thinking and other cognitive functions that affects daily activities. It can be caused by several diseases, including Alzheimer's disease.

Q4. How can falls be prevented in elderly patients?

Answer: Fall prevention includes assessing fall risk, maintaining safe footwear, providing walking assistance and keeping the environment free of obstacles. Adequate lighting and access to mobility aids are also important.

Q5. Why is nutrition important in elderly patients?

Answer: Good nutrition helps maintain muscle strength, immunity, bone health and recovery from illness. Nurses should monitor food intake, hydration, swallowing difficulties and signs of malnutrition.

Junior Level

Q1. How would you care for an elderly patient with dementia who becomes confused?

Answer: I would maintain a calm environment, use simple communication and provide familiar routines. I would assess for possible causes of sudden confusion, such as infection, pain or medication effects, and report changes to the doctor.

Q2. What is delirium, and how is it different from dementia?

Answer: Delirium is an acute disturbance in attention and awareness that usually develops over hours or days. Dementia generally develops gradually and causes progressive cognitive decline. New confusion in an older patient requires urgent assessment.

Q3. How would you prevent pressure injuries in bedridden elderly patients?

Answer: I assess skin condition and pressure-injury risk, reposition the patient according to the care plan and use suitable pressure-relieving surfaces. I maintain skin hygiene, adequate nutrition and hydration, and document skin changes.

Q4. How do you support an elderly patient with mobility problems?

Answer: I assess mobility and fall risk, assist with transfers and walking, use prescribed mobility aids and encourage safe exercises. I coordinate with physiotherapists and promote independence within the patient's abilities.

Q5. How do you manage an elderly patient taking multiple medications?

Answer: I review the medication chart, verify doses and administration times, observe for adverse reactions and report possible interactions or duplicate prescriptions. I also encourage medication reconciliation during admission and discharge.

Senior Level

Q1. How would you manage an elderly patient with sudden confusion and reduced consciousness?

Answer: I would treat this as acute clinical deterioration and initiate ABCDE assessment. I would check vital signs and blood glucose, assess neurological status and look for causes such as infection, stroke or medication effects. I would immediately escalate and coordinate investigations and treatment.

Q2. How would you manage an elderly patient with multiple chronic illnesses?

Answer: I would coordinate an individualized care plan that considers comorbidities, medication burden, mobility, nutrition and cognitive status. I would work with physicians, pharmacists, physiotherapists and caregivers to prevent complications and maintain functional independence.

Q3. How would you handle a patient who refuses personal care?

Answer: I would respect the patient's dignity and decision-making capacity, explore the reason for refusal and explain the importance of care. I would offer choices and alternative timing, involve caregivers when appropriate and document persistent refusal.

Q4. How would you reduce hospital readmissions among elderly patients?

Answer: I would support comprehensive discharge planning, medication reconciliation, caregiver education and follow-up arrangements. I would ensure mobility needs, nutrition, wound care and warning signs are addressed before discharge.

Q5. How would you manage an elderly patient at high risk of falls?

Answer: I would perform a multifactorial fall-risk assessment, review medications with the clinical team, implement environmental precautions and arrange mobility support. I would educate caregivers, reassess risk regularly and review every fall or near-miss incident.

8. Trauma Nurse / Emergency Nurse

Fresher Level

Q1. What is the role of an emergency nurse?

Answer: An emergency nurse assesses patients arriving with acute illness or injuries, identifies urgent conditions and assists with immediate treatment. The role includes triage, monitoring, emergency procedures and coordination with the medical team.

Q2. What is the ABCDE approach?

Answer: ABCDE stands for Airway, Breathing, Circulation, Disability and Exposure. It is a systematic approach to identifying and managing immediate life-threatening problems.

Q3. What is triage?

Answer: Triage is the process of assessing and prioritizing patients according to the severity and urgency of their conditions. It helps ensure that patients with life-threatening conditions receive immediate attention.

Q4. What is shock?

Answer: Shock is a life-threatening condition in which inadequate tissue perfusion prevents organs from receiving sufficient oxygen and nutrients. Causes include severe bleeding, infection, cardiac dysfunction and allergic reactions.

Q5. What is CPR?

Answer: Cardiopulmonary resuscitation is an emergency procedure used when a person has cardiac arrest. It involves chest compressions and, when indicated, ventilation to maintain circulation and oxygen delivery until advanced treatment is available.

Junior Level

Q1. A road-traffic accident patient arrives with severe bleeding. What will you do?

Answer: I would initiate trauma assessment, call for assistance and control external bleeding using direct pressure or appropriate hemorrhage-control measures. I would assess airway, breathing and circulation, protect the cervical spine when indicated and prepare for urgent resuscitation.

Q2. How would you assess a patient with suspected head injury?

Answer: I would assess airway, breathing, circulation, level of consciousness, pupil response and limb movement. I would monitor for vomiting, seizures and neurological deterioration, maintain spinal precautions when indicated and escalate concerning findings.

Q3. What is the difference between hypovolemic and cardiogenic shock?

Answer: Hypovolemic shock occurs due to reduced circulating blood or fluid volume, such as severe bleeding. Cardiogenic shock occurs when the heart cannot pump enough blood to meet the body's needs, often due to severe cardiac dysfunction.

Q4. What would you do if a patient develops anaphylaxis?

Answer: I would recognize symptoms such as breathing difficulty, swelling and hypotension, activate emergency assistance and follow the anaphylaxis protocol. I would assist with immediate intramuscular adrenaline administration according to protocol and monitor airway, breathing and circulation.

Q5. How do you manage a patient with suspected spinal injury?

Answer: I would minimize unnecessary movement, maintain appropriate spinal alignment and support airway and breathing. I would use approved immobilization procedures when indicated and coordinate urgent medical assessment and imaging.

Senior Level

Q1. How would you coordinate the management of a patient with multiple traumatic injuries?

Answer: I would activate the trauma response and coordinate a structured primary survey using ABCDE. I would prioritize airway compromise and uncontrolled hemorrhage, ensure rapid access to resuscitation resources and coordinate imaging, blood products and specialist involvement with the trauma team.

Q2. How would you manage a mass-casualty incident in the emergency department?

Answer: I would activate the hospital disaster plan, establish triage and support patient-flow management. I would coordinate staff, equipment and treatment areas, maintain communication with emergency services and direct resources according to triage priority.

Q3. How would you identify and manage impending respiratory failure?

Answer: I would recognize increasing respiratory effort, altered mental status, exhaustion, abnormal respiratory patterns and worsening oxygenation. I would immediately escalate, support airway and breathing, prepare emergency equipment and assist the medical team with advanced respiratory support.

Q4. How would you manage a patient with suspected internal hemorrhage?

Answer: I would monitor for tachycardia, hypotension, pallor, abdominal distension and altered mental status. I would activate the appropriate emergency pathway, prepare for vascular access and blood-product support, assist with investigations and monitor for deterioration.

Q5. How would you lead a cardiac-arrest response in the emergency department?

Answer: I would ensure immediate activation of the resuscitation team, assign clear roles and support high-quality CPR and protocol-based treatment. I would ensure medication and defibrillator readiness, document the timeline and support post-resuscitation care and debriefing.

9. Surgical Nurse

Fresher Level

Q1. What is surgical nursing?

Answer: Surgical nursing involves caring for patients before, during and after surgical procedures. It includes patient preparation, monitoring, wound care, pain management and recovery support.

Q2. What is preoperative care?

Answer: Preoperative care includes reviewing patient history, verifying identity and surgical consent, checking allergies, reviewing investigations and preparing the patient for surgery according to hospital instructions.

Q3. What is postoperative care?

Answer: Postoperative care involves monitoring vital signs, consciousness, pain, surgical wounds, bleeding and fluid balance. It also includes assisting with mobility, nutrition and recovery.

Q4. What are common signs of wound infection?

Answer: Common signs include redness, swelling, warmth, increasing pain, pus discharge and fever. These findings should be reported and assessed promptly.

Q5. Why is informed consent important before surgery?

Answer: Informed consent ensures that the patient understands the proposed procedure, potential risks, benefits and available alternatives. The nurse verifies that consent documentation is present and escalates concerns about understanding or voluntariness.

Junior Level

Q1. What would you monitor during the first few hours after surgery?

Answer: I would closely monitor airway, breathing, circulation, vital signs and level of consciousness. I would also assess pain, surgical-site bleeding, wound condition, drains and fluid balance.

Q2. What would you do if a postoperative patient develops wound bleeding?

Answer: I would assess the amount of bleeding, check vital signs and immediately inform the surgical team. I would follow prescribed wound-management and emergency procedures, maintain patient safety and document findings.

Q3. How do you manage postoperative pain?

Answer: I assess pain intensity and characteristics, administer prescribed analgesics and provide positioning and other comfort measures. I reassess pain relief and monitor for medication-related adverse effects.

Q4. Why is early mobilization important after surgery?

Answer: Early mobilization, when clinically appropriate, helps reduce complications such as venous thromboembolism, respiratory problems, muscle weakness and constipation. It also supports recovery and functional independence.

Q5. What is the purpose of a surgical drain?

Answer: A surgical drain removes blood, fluid or other secretions from an operative site. Nurses monitor the amount, colour and character of drainage and maintain the drain according to the prescribed care plan.

Senior Level

Q1. How would you manage suspected postoperative hemorrhage?

Answer: I would immediately assess ABCs, vital signs, surgical-site bleeding and signs of poor perfusion. I would activate emergency assistance, notify the surgeon and support prescribed resuscitation, investigations and blood-product management. I would continue close monitoring and documentation.

Q2. How would you manage suspected postoperative pulmonary embolism?

Answer: I would recognize warning signs such as sudden breathlessness, chest pain, tachycardia and reduced oxygen saturation. I would activate emergency support, assess ABCs, provide prescribed oxygen support and coordinate urgent medical evaluation and treatment.

Q3. How do you reduce surgical-site infections in your department?

Answer: I ensure compliance with preoperative preparation, appropriate antimicrobial prophylaxis, sterile dressing practices and hand hygiene. I monitor wound infections, review audit findings and work with infection-control staff to improve prevention.

Q4. How would you coordinate care for a patient undergoing major abdominal surgery?

Answer: I would ensure preoperative readiness and coordinate with anesthesia and surgical teams. Postoperatively, I would monitor hemodynamic stability, respiratory status, wound condition, bowel function, pain and fluid balance.

Q5. How would you manage a surgical patient who develops sudden deterioration on the ward?

Answer: I would activate the emergency response, initiate ABCDE assessment and identify immediate threats. I would coordinate rapid medical review, provide care within my scope, prepare emergency equipment and ensure ongoing monitoring and documentation.

10. OPD Nurse

Fresher Level

Q1. What is an Outpatient Department?

Answer: The Outpatient Department is a hospital section where patients receive consultations, examinations, diagnosis and treatment without being admitted as inpatients.

Q2. What are the responsibilities of an OPD nurse?

Answer: Responsibilities include checking vital signs, assisting doctors during examinations, preparing clinical equipment, supporting patient flow, providing health education and maintaining records.

Q3. Why is patient identification important in OPD?

Answer: Correct patient identification prevents errors in consultation, investigations, procedures and medication administration. Approved patient identifiers should be verified before care.

Q4. What is the importance of maintaining patient confidentiality?

Answer: Confidentiality protects personal and medical information from unauthorized access or disclosure. Nurses should discuss patient information only with authorized healthcare professionals and relevant individuals.

Q5. What is patient registration?

Answer: Patient registration records patient identity, contact details and relevant administrative information to establish an accurate hospital record.

Junior Level

Q1. How would you manage a crowded OPD?

Answer: I would follow the hospital's patient-flow procedure, prioritize urgent cases, coordinate with doctors and communicate clearly with waiting patients. I would ensure emergency symptoms are not overlooked because of routine queues.

Q2. What would you do if a patient develops chest pain while waiting for consultation?

Answer: I would immediately alert the emergency team, assess ABCs and vital signs and arrange urgent evaluation. I would not allow the patient to remain in the routine consultation queue.

Q3. How do you prepare a patient for a minor OPD procedure?

Answer: I verify identity, explain the procedure within my responsibilities, check relevant allergies and documentation, prepare equipment and maintain aseptic technique. I monitor the patient during and after the procedure.

Q4. What would you do if a patient becomes angry due to a long waiting time?

Answer: I would listen calmly, acknowledge the concern and explain the waiting process without making unrealistic promises. I would assess whether the patient's condition has changed and inform appropriate staff if support is needed.

Q5. How do you maintain accurate OPD documentation?

Answer: I record patient assessments, vital signs, procedures, medications and relevant observations promptly. I ensure entries are accurate, dated and authenticated according to hospital policy.

Senior Level

Q1. How would you manage patient flow in a high-volume OPD?

Answer: I would review appointment patterns, staffing and patient-acuity trends. I would coordinate triage, registration and consultation areas, identify bottlenecks and implement improvements while maintaining emergency access and confidentiality.

Q2. How would you handle an OPD patient suspected of having a stroke?

Answer: I would immediately activate the stroke or emergency pathway, establish the time the patient was last known well, assess neurological warning signs and vital parameters, and arrange urgent medical evaluation. I would not delay escalation for routine administration.

Q3. How would you manage a suspected infectious-disease case in OPD?

Answer: I would identify symptoms and exposure risks, arrange appropriate separation or isolation precautions and use required PPE. I would inform infection-control staff and follow reporting, referral and testing procedures.

Q4. How would you improve OPD patient satisfaction?

Answer: I would review complaints and waiting times, improve communication, coordinate staff responsibilities and strengthen patient education. I would monitor service-quality indicators and implement corrective actions.

Q5. How would you supervise OPD nursing staff?

Answer: I would allocate duties according to workload and competency, monitor documentation and infection-control compliance, provide clinical guidance and evaluate performance through audits and patient feedback.

11. Ward Nurse

Fresher Level

Q1. What are the main responsibilities of a ward nurse?

Answer: A ward nurse provides bedside care, administers prescribed medications, monitors patient progress, assists doctors and maintains nursing records. The nurse also supports hygiene, nutrition, mobility and discharge preparation.

Q2. What is a nursing care plan?

Answer: A nursing care plan identifies patient problems, expected outcomes and nursing interventions. It helps organize individualized care and evaluate progress.

Q3. What is patient handover?

Answer: Patient handover is the transfer of important clinical information between healthcare workers during a shift change or transfer. It supports continuity of care and reduces communication errors.

Q4. What is intake and output charting?

Answer: It is the systematic recording of fluids taken by the patient and fluids eliminated through urine, drains and other measurable routes. It helps assess hydration and fluid balance.

Q5. Why is patient positioning important?

Answer: Correct positioning improves comfort, supports breathing and circulation and reduces pressure injuries. It is important for patients with limited mobility or specific medical conditions.

Junior Level

Q1. How would you manage multiple patients during your shift?

Answer: I would assess each patient's clinical condition, prioritize urgent care, organize medication and treatment schedules and delegate appropriate activities. I would regularly reassess patients and communicate changes to the medical team.

Q2. What would you do if a patient develops sudden breathlessness in the ward?

Answer: I would assess airway, breathing and circulation, check oxygen saturation and vital signs, call for emergency assistance and support the patient according to protocol. I would continue monitoring until the medical team takes over.

Q3. How would you care for a patient with a urinary catheter?

Answer: I maintain hand hygiene, provide catheter hygiene, keep the drainage system closed and position the urine bag below bladder level. I monitor urine output and signs of infection and avoid unnecessary catheter manipulation.

Q4. What is pressure-injury staging?

Answer: Pressure-injury staging classifies tissue damage according to severity and depth. Categories include Stages 1–4, unstageable pressure injury and deep tissue pressure injury.

Q5. What information should be included in patient handover?

Answer: Handover should include patient identity, diagnosis, current condition, vital signs, allergies, medications, treatment plans, risks, pending investigations and important changes during the shift.

Senior Level

Q1. How would you manage a ward experiencing a sudden increase in admissions?

Answer: I would assess available beds, staffing and patient acuity, coordinate admissions with the nursing supervisor and ensure safe staff allocation. I would prioritize urgent care, maintain infection-control precautions and monitor workload and patient safety.

Q2. How would you handle a patient with suspected hospital-acquired infection?

Answer: I would identify clinical warning signs, implement appropriate precautions, inform the treating team and infection-control department, and assist with prescribed investigations. I would review possible exposure sources and ensure documentation and surveillance.

Q3. How do you ensure effective shift management?

Answer: I prepare a structured shift plan, assign responsibilities according to competency and monitor completion of essential nursing activities. I conduct rounds, address emerging clinical needs and ensure accurate handover.

Q4. How would you manage a serious patient complaint involving nursing negligence?

Answer: I would ensure immediate patient safety, report the allegation through the appropriate management process and preserve accurate records. I would support an objective investigation, cooperate with hospital leadership and implement corrective actions based on findings.

Q5. How would you improve patient safety in a general ward?

Answer: I would strengthen medication safety, patient identification, fall prevention, pressure-injury prevention and handover standards. I would use audits, incident reviews and staff training to identify risks and measure improvement.

12. GI Nurse

Fresher Level

Q1. What is gastrointestinal nursing?

Answer: Gastrointestinal nursing focuses on patients with disorders of the digestive system, including the esophagus, stomach, intestines, liver, pancreas and gallbladder.

Q2. What is endoscopy?

Answer: Endoscopy uses a flexible instrument with a camera to examine the inside of the digestive tract. It can also be used for certain diagnostic and therapeutic procedures.

Q3. What is the difference between gastroscopy and colonoscopy?

Answer: Gastroscopy examines the upper digestive tract, including the esophagus, stomach and duodenum. Colonoscopy examines the large intestine and rectum.

Q4. What are common symptoms of gastrointestinal disorders?

Answer: Common symptoms include abdominal pain, nausea, vomiting, diarrhea, constipation, bloating and gastrointestinal bleeding. Persistent or severe symptoms require medical assessment.

Q5. Why is fasting required before endoscopy?

Answer: Fasting helps reduce stomach contents and the risk of aspiration during sedation or the procedure. The fasting duration must follow the specific procedure and anesthesia instructions.

Junior Level

Q1. What would you monitor after an upper GI endoscopy?

Answer: I would monitor vital signs, consciousness, breathing and pain, and observe for bleeding or other complications. I would follow post-sedation instructions and ensure oral intake resumes only when swallowing safety is established.

Q2. What are warning signs of gastrointestinal bleeding?

Answer: Warning signs include vomiting blood, black tarry stools, fresh rectal bleeding, dizziness, tachycardia and hypotension. I would report these findings immediately and assess hemodynamic condition.

Q3. How would you prepare a patient for colonoscopy?

Answer: I would verify procedure instructions, check prescribed bowel preparation and dietary restrictions, review relevant medications and allergies, and confirm documentation. I would monitor preparation tolerance and report incomplete cleansing or concerning symptoms.

Q4. What is an ostomy, and how do you care for it?

Answer: An ostomy is a surgically created opening that allows body waste to leave through the abdominal wall. Care includes assessing stoma colour, skin condition, output and appliance fit while teaching hygiene and pouch management.

Q5. How would you care for a patient with persistent vomiting?

Answer: I would assess hydration, vital signs, abdominal symptoms and vomiting characteristics. I would maintain aspiration precautions, monitor fluid balance, administer prescribed treatment and report signs of dehydration or obstruction.

Senior Level

Q1. How would you manage suspected upper GI bleeding with hemodynamic instability?

Answer: I would activate emergency support, assess ABCs and recognize shock. I would assist with vascular access, prescribed fluid and blood-product resuscitation, urgent investigations and endoscopic preparation while monitoring vital signs and perfusion.

Q2. How would you manage a patient with hepatic encephalopathy?

Answer: I would assess consciousness, neurological changes, airway safety and fall risk. I would coordinate prescribed treatment, monitor response, identify possible precipitating factors with the medical team and educate caregivers about recurrence prevention.

Q3. How would you manage a patient following a major GI procedure?

Answer: I would monitor hemodynamic status, abdominal findings, pain, bleeding, bowel function and fluid balance. I would identify signs of perforation, infection or other complications and escalate immediately.

Q4. How would you supervise GI procedure-room nursing staff?

Answer: I would ensure procedure readiness, infection prevention, equipment competency, patient identification and appropriate sedation monitoring. I would conduct competency assessments and audit specimen handling and procedure documentation.

Q5. How would you improve safety in a GI endoscopy unit?

Answer: I would strengthen pre-procedure screening, consent verification, sedation monitoring, scope reprocessing, emergency preparedness and post-procedure observation. I would review adverse events and audit compliance with current infection-control standards.

13. Camp Nurse

Fresher Level

Q1. What is the role of a camp nurse?

Answer: A camp nurse provides basic healthcare during community medical camps, workplace health programs and outreach activities. Responsibilities include screening, first aid, patient education and referral.

Q2. What activities are performed during a medical camp?

Answer: Activities may include registration, blood pressure and glucose screening, basic health assessments, first aid, health education and referral of patients who need further evaluation.

Q3. How do you measure blood glucose using a glucometer?

Answer: I verify the patient's identity, perform hand hygiene, prepare the glucometer and test strip, obtain an appropriate blood sample using a safe lancet and record the result. I follow device instructions and infection-control procedures.

Q4. What is first aid?

Answer: First aid is immediate assistance given to a person who is injured or suddenly ill before definitive medical treatment is available. It aims to preserve life, prevent further harm and support recovery.

Q5. Why is health education important in community camps?

Answer: Health education helps people understand disease prevention, nutrition, hygiene, vaccination and early identification of health problems. It encourages healthy behaviours and timely medical consultation.

Junior Level

Q1. What would you do if a person faints during a medical camp?

Answer: I would assess responsiveness and breathing, protect the person from injury and call for assistance. I would position them safely according to cause and condition, monitor vital signs and arrange urgent evaluation when necessary.

Q2. How would you manage a person with very high blood pressure during screening?

Answer: I would repeat the measurement using correct technique and assess symptoms such as chest pain, severe headache, breathlessness or neurological changes. I would arrange urgent medical evaluation when indicated and avoid independently prescribing medication.

Q3. How do you maintain infection control in an outdoor medical camp?

Answer: I maintain hand hygiene, use appropriate PPE, disinfect reusable equipment and segregate biomedical waste correctly. I ensure safe sharps disposal and maintain a clean examination area.

Q4. How would you manage a person with a minor wound?

Answer: I would assess the wound, perform appropriate cleaning and dressing using aseptic technique, and evaluate the need for further medical assessment and tetanus protection. I would refer deep, contaminated or infected wounds.

Q5. How do you identify patients who need hospital referral?

Answer: I identify warning signs such as chest pain, severe breathlessness, altered consciousness, uncontrolled bleeding, stroke symptoms and severe dehydration. I arrange timely referral and communicate findings to the receiving facility.

Senior Level

Q1. How would you organize a large community health camp?

Answer: I would plan staffing, patient-flow arrangements, screening stations, equipment, supplies and referral pathways. I would establish infection-control procedures, assign responsibilities and monitor service delivery and documentation.

Q2. How would you respond to a medical emergency during a camp with limited equipment?

Answer: I would activate emergency services, assess ABCs, provide immediate first aid and use available emergency equipment within my competency. I would arrange rapid transport to an appropriate facility and communicate essential patient information.

Q3. How would you manage a suspected infectious-disease outbreak at a camp?

Answer: I would implement appropriate separation and infection-control measures, inform public-health authorities and follow reporting requirements. I would coordinate testing, referrals and health education with the responsible public-health team.

Q4. How would you evaluate the effectiveness of a medical camp?

Answer: I would review the number screened, conditions identified, referrals completed, educational activities and follow-up outcomes. I would use findings to improve future planning and resource allocation.

Q5. How would you supervise nurses and volunteers during a health camp?

Answer: I would define roles, provide pre-camp training and ensure activities remain within each person's scope. I would monitor patient safety, manage workflow, address operational problems and review performance after the camp.

14. Physician Assistant Nurse

Scope note: Physician assistants and registered nurses are distinct professions. These questions assume a nurse in a physician-support role and do not imply independent physician practice.

Fresher Level

Q1. What is the role of a nurse assisting a physician?

Answer: The nurse supports patient assessment, prepares patients for examinations, monitors vital signs and assists with clinical procedures within authorized scope. The role also includes documentation and coordination of care.

Q2. How do you prepare a patient for a medical examination?

Answer: I verify the patient's identity, explain the examination, obtain required vital signs and prepare necessary equipment. I maintain privacy and ensure relevant medical information is available.

Q3. What is differential diagnosis?

Answer: Differential diagnosis is the process of considering different possible conditions that could explain a patient's symptoms. The physician evaluates these possibilities using history, clinical findings and investigations.

Q4. Why is taking patient history important?

Answer: Patient history provides information about symptoms, previous illnesses, allergies, medications and family history. It helps the physician understand the condition and plan appropriate investigations and treatment.

Q5. What is informed consent?

Answer: Informed consent is the patient's voluntary agreement to a procedure or treatment after receiving information about purpose, risks, benefits and alternatives. The responsible clinician must ensure the process meets legal and hospital requirements.

Junior Level

Q1. How would you assist a physician during a minor procedure?

Answer: I would prepare equipment, verify patient identity, check relevant allergies and maintain aseptic technique. I would assist within my competency, monitor the patient and document nursing activities.

Q2. What would you do if you identify an apparently incorrect medication order?

Answer: I would not administer the medication until the concern is clarified. I would verify the prescription with the authorized prescriber, review allergies and follow medication-safety procedures.

Q3. How do you assist in managing a patient with diabetes during consultation?

Answer: I would obtain relevant measurements, review available glucose records, ask about symptoms and medication adherence, and communicate findings to the physician. I would provide approved education and assist with the prescribed care plan.

Q4. What would you do if a patient experiences an adverse reaction during treatment?

Answer: I would stop or withhold further administration when appropriate under protocol, assess ABCs and vital signs, and immediately alert the responsible physician. I would assist with emergency management and document the reaction.

Q5. How do you ensure continuity of care after a physician consultation?

Answer: I ensure treatment instructions, investigations, medication changes and follow-up recommendations are accurately documented. I communicate relevant information to the care team and clarify incomplete orders.

Senior Level

Q1. How would you coordinate care for a complex patient with multiple conditions?

Answer: I would organize clinical information, identify immediate nursing concerns and coordinate communication between the physician and relevant specialists. I would ensure monitoring, investigations, medications and follow-up plans are documented and implemented within my scope.

Q2. How would you manage disagreement between an order and a documented allergy?

Answer: I would immediately raise the discrepancy with the prescriber and pharmacist and withhold the potentially unsafe medication until clarified. I would ensure the allergy record is accurate and document the resolution.

Q3. How would you ensure safe delegation of clinical tasks?

Answer: I would consider the patient's condition, task complexity and the competency and authorization of the person receiving the task. I would clarify expected outcomes, provide appropriate supervision and ensure escalation when needed.

Q4. How would you improve clinical documentation in a physician-support unit?

Answer: I would establish standardized documentation procedures, verify completeness of patient records and monitor accuracy of clinical entries. I would conduct audits and train staff on confidentiality and documentation requirements.

Q5. How would you respond if a patient deteriorates while waiting for a physician?

Answer: I would immediately assess the patient, activate emergency assistance and initiate ABCDE-based assessment. I would provide nursing interventions within my competency, arrange urgent physician review and ensure the patient is not left unattended.

15. Medical Nurse

Fresher Level

Q1. What is medical nursing?

Answer: Medical nursing focuses on patients with illnesses generally managed through medical rather than surgical treatment. It includes monitoring, medication administration, patient education and chronic-disease care.

Q2. What are common diseases treated in a medical ward?

Answer: Common conditions include diabetes, hypertension, pneumonia, asthma, heart failure, kidney disease and infectious diseases. Patients may require medication, oxygen support, monitoring and lifestyle education.

Q3. What is hypoglycemia?

Answer: Hypoglycemia is abnormally low blood glucose, commonly defined as below 70 mg/dL in many adult diabetes-care settings. Symptoms may include sweating, trembling, hunger, confusion and dizziness.

Q4. What is hypertension?

Answer: Hypertension is persistently elevated blood pressure. It increases the risk of cardiovascular disease, stroke and kidney damage and requires appropriate monitoring and management.

Q5. What is medication adherence?

Answer: Medication adherence means taking prescribed medicines correctly and consistently. It helps control disease, prevent complications and improve treatment outcomes.

Junior Level

Q1. How would you care for a patient with uncontrolled diabetes?

Answer: I would monitor blood glucose as prescribed, assess for symptoms of hypo- and hyperglycemia, administer ordered medication and monitor food intake and hydration. I would report abnormal results and reinforce diabetes education.

Q2. What would you do if a patient develops hypoglycemia?

Answer: I would assess consciousness and confirm blood glucose according to protocol. If conscious and able to swallow safely, I would provide recommended fast-acting carbohydrate treatment. If unconscious or unable to swallow, I would activate emergency support and assist with prescribed IV glucose or glucagon treatment.

Q3. How do you monitor a patient with pneumonia?

Answer: I monitor respiratory rate, oxygen saturation, temperature, breathlessness, sputum and general condition. I administer prescribed treatment, support appropriate hydration and report worsening respiratory function.

Q4. What is fluid overload, and what are its signs?

Answer: Fluid overload occurs when the body retains more fluid than it can safely manage. Signs include edema, rapid weight gain, breathlessness, lung crackles and raised blood pressure.

Q5. How would you care for a patient with chronic kidney disease?

Answer: I monitor blood pressure, fluid balance, urine output, weight and relevant laboratory results. I follow prescribed dietary and fluid restrictions and educate the patient about medication adherence and follow-up.

Senior Level

Q1. How would you manage a patient with acute pulmonary edema?

Answer: I would activate emergency assistance, assess ABCs and monitor oxygenation and hemodynamic status. I would position the patient appropriately, assist with prescribed respiratory support and medications, and closely monitor response and fluid balance.

Q2. How would you manage a patient with diabetic ketoacidosis?

Answer: I would coordinate urgent medical care, monitor vital signs, neurological status, blood glucose, electrolytes and fluid balance, and assist with prescribed IV fluids, insulin and electrolyte management. I would monitor for complications such as hypokalemia and cerebral edema.

Q3. How would you manage a patient with acute kidney injury?

Answer: I would monitor urine output, fluid balance, blood pressure, electrolytes and renal function. I would review contributing factors with the medical team, avoid unauthorized nephrotoxic medication administration and escalate deterioration.

Q4. How would you supervise nurses caring for complex medical patients?

Answer: I would allocate patients according to acuity and competency, ensure treatment and monitoring plans are understood and review documentation. I would provide clinical guidance, support escalation and evaluate nursing outcomes.

Q5. How would you coordinate discharge for a patient with multiple chronic diseases?

Answer: I would coordinate medication reconciliation, follow-up appointments, dietary and lifestyle education and caregiver involvement. I would ensure the patient understands warning signs and has a clear plan for accessing care after discharge.

16. Orthopedic Nurse

Fresher Level

Q1. What is orthopedic nursing?

Answer: Orthopedic nursing focuses on patients with conditions affecting bones, joints, muscles, ligaments and the skeletal system. It includes fracture care, postoperative care, mobility assistance and pain management.

Q2. What is a fracture?

Answer: A fracture is a partial or complete break in the continuity of a bone. It may result from trauma, repetitive stress or weakened bones.

Q3. What is a plaster cast?

Answer: A plaster cast is a rigid support applied to an injured limb to immobilize and protect a bone or joint during healing. Nurses monitor circulation, sensation, movement and skin condition.

Q4. What is traction?

Answer: Traction applies a controlled pulling force to maintain alignment, reduce muscle spasms or support fracture management. It must be maintained according to the prescribed setup.

Q5. Why is neurovascular assessment important?

Answer: Neurovascular assessment checks circulation, sensation and movement in an affected limb. It helps identify complications such as nerve compression or impaired blood flow.

Junior Level

Q1. What would you monitor in a patient with a plaster cast?

Answer: I would assess pain, swelling, skin colour, temperature, sensation, movement and distal pulses. I would report increasing pain, numbness, excessive swelling or reduced circulation immediately.

Q2. What is compartment syndrome?

Answer: Compartment syndrome is a serious condition in which increased pressure within a closed muscle compartment compromises circulation and tissue function. Severe pain, especially with passive stretch, is an important warning sign and requires urgent evaluation.

Q3. How would you care for a patient after hip replacement surgery?

Answer: I would monitor vital signs, pain, wound condition and limb neurovascular status. I would follow prescribed movement precautions, assist with early mobilization and monitor for infection and venous thromboembolism.

Q4. How do you prevent deep vein thrombosis in orthopedic patients?

Answer: I encourage early mobilization when permitted, use prescribed compression devices or stockings and administer anticoagulants as ordered. I monitor for unilateral swelling, pain and other signs of venous thrombosis.

Q5. How would you assist a patient using crutches?

Answer: I would ensure correct crutch height and hand positioning, explain the prescribed weight-bearing status and demonstrate the correct walking technique. I would supervise practice and assess balance and safety.

Senior Level

Q1. How would you manage suspected compartment syndrome?

Answer: I would immediately notify the orthopedic surgeon, assess pain, sensation, movement, swelling and circulation, and document changes. I would avoid delaying definitive treatment and follow emergency instructions regarding limb positioning and constrictive devices.

Q2. How would you manage a patient with an open fracture?

Answer: I would prioritize trauma assessment, control bleeding, cover the wound with a sterile dressing and avoid manipulating exposed bone. I would maintain appropriate immobilization, monitor circulation and coordinate urgent surgical assessment and prescribed antimicrobial and tetanus management.

Q3. How would you coordinate care after total knee replacement?

Answer: I would coordinate pain management, wound assessment, neurovascular monitoring, mobility training and thrombosis prevention. I would work with physiotherapists and the surgical team to ensure safe rehabilitation and discharge planning.

Q4. How would you manage an orthopedic patient who develops calf swelling and breathlessness?

Answer: I would suspect possible venous thromboembolism with pulmonary embolism, activate emergency assistance and assess ABCs. I would monitor vital signs, provide prescribed support and coordinate urgent medical investigation and treatment.

Q5. How would you supervise orthopedic nursing staff?

Answer: I would assess competency in neurovascular observations, cast care, traction, postoperative monitoring and mobility assistance. I would allocate patients appropriately, review complications and conduct training and audits.

17. Neuro Nurse

Fresher Level

Q1. What is neurological nursing?

Answer: Neurological nursing focuses on patients with disorders affecting the brain, spinal cord and peripheral nervous system. It includes neurological assessment, monitoring and rehabilitation support.

Q2. What is the Glasgow Coma Scale?

Answer: GCS evaluates consciousness through eye opening, verbal response and motor response. Its score ranges from 3 to 15, with lower scores indicating more severe impairment of consciousness.

Q3. What are the warning signs of stroke?

Answer: FAST signs are facial drooping, arm weakness, speech difficulty and time to seek emergency treatment. Sudden visual changes, balance problems and severe unexplained headache can also indicate stroke.

Q4. What is a seizure?

Answer: A seizure is a sudden episode of abnormal electrical activity in the brain that can cause changes in movement, awareness, sensation or behaviour. Seizures have different causes and presentations.

Q5. Why is pupil assessment important?

Answer: Pupil size, equality and reaction to light help identify possible neurological deterioration. Changes may indicate pressure or injury affecting the brain and require prompt assessment.

Junior Level

Q1. How would you care for a patient experiencing a seizure?

Answer: I would protect the patient from injury, remove nearby hazards, protect the head and note seizure duration. I would not restrain the patient or place anything in the mouth. I would maintain airway safety and seek medical assistance.

Q2. What would you do if a stroke patient develops difficulty swallowing?

Answer: I would withhold oral food, fluids and medication until swallowing safety is assessed. I would inform the medical team, follow the swallowing-assessment pathway and use the prescribed alternative route for essential medication or nutrition.

Q3. What are signs of increased intracranial pressure?

Answer: Possible signs include worsening headache, vomiting, altered consciousness, pupil changes and neurological deficits. Severe deterioration may produce abnormal breathing and cardiovascular changes.

Q4. How do you monitor a patient after a head injury?

Answer: I monitor consciousness, GCS, pupil response, limb movement, vital signs and symptoms such as headache or vomiting. I report neurological deterioration immediately.

Q5. What is the purpose of neurological observation charts?

Answer: They record changes in consciousness, pupils, motor function and other relevant parameters. They help identify deterioration early and support timely intervention.

Senior Level

Q1. How would you manage a patient with suspected acute stroke?

Answer: I would activate the stroke pathway, establish the time the patient was last known well and assess ABCs and neurological status. I would coordinate urgent brain imaging, blood glucose assessment and specialist review without delaying time-sensitive treatment.

Q2. How would you manage suspected increased intracranial pressure?

Answer: I would immediately escalate the concern, assess neurological status and support airway and oxygenation. I would follow prescribed positioning and monitoring measures, minimize unnecessary stimulation and prepare for urgent investigations and specialist treatment.

Q3. How would you manage a patient following neurosurgery?

Answer: I would monitor consciousness, pupils, motor function, vital signs, pain, wound condition and drain output. I would assess for bleeding, seizures, infection and neurological deterioration and escalate abnormalities promptly.

Q4. How would you manage status epilepticus?

Answer: I would activate the emergency response, protect airway and breathing, monitor oxygenation and circulation and assist with prescribed emergency anticonvulsant treatment. I would document seizure activity and treatment response and prepare for further escalation.

Q5. How would you lead a neurological nursing team?

Answer: I would establish standards for neurological assessment, observation frequency, escalation and documentation. I would supervise competency, coordinate multidisciplinary rehabilitation and review patient outcomes to improve care quality.

18. Psychiatric Nurse

Fresher Level

Q1. What is psychiatric nursing?

Answer: Psychiatric nursing focuses on the care of people experiencing mental health conditions. It includes assessment, therapeutic communication, medication support, safety and recovery-oriented care.

Q2. What is therapeutic communication?

Answer: Therapeutic communication uses active listening, empathy, respect and appropriate questioning to support patients. It helps establish trust and understand patient needs.

Q3. What are common symptoms of depression?

Answer: Symptoms may include persistent sadness, loss of interest, sleep disturbances, fatigue, appetite changes, poor concentration and feelings of worthlessness. Some patients may also experience suicidal thoughts.

Q4. What is schizophrenia?

Answer: Schizophrenia is a serious mental health disorder that can affect thinking, perception, emotions and behaviour. Symptoms may include hallucinations, delusions and disorganized thinking.

Q5. Why is confidentiality important in psychiatric nursing?

Answer: Confidentiality protects privacy and encourages open communication. Information may need to be shared with authorized professionals when necessary for treatment or safety, following applicable law and policy.

Junior Level

Q1. How would you manage an aggressive patient?

Answer: I would maintain a safe distance, use calm and respectful communication and avoid confrontation. I would reduce environmental stimulation, seek assistance from trained staff and follow de-escalation and safety procedures.

Q2. What would you do if a patient reports hearing voices?

Answer: I would listen without judgment, acknowledge distress and assess whether the voices direct harmful actions. I would inform the mental-health team and maintain appropriate safety observations.

Q3. How do you care for a patient experiencing a panic attack?

Answer: I would remain calm, provide a safe environment and communicate using short, clear statements. I would assess physical symptoms, help the patient use appropriate coping techniques and seek medical evaluation if symptoms are severe or atypical.

Q4. What would you do if a psychiatric patient refuses medication?

Answer: I would explore the reason for refusal, assess understanding and explain the medication's purpose and potential effects. I would respect legal rights, inform the responsible clinician and document the refusal according to policy.

Q5. How would you assess a patient at risk of self-harm?

Answer: I would communicate directly and nonjudgmentally about thoughts, intent, plans and immediate access to means. I would promptly inform the mental-health team, follow the risk-management plan and maintain appropriate observation and environmental safety.

Senior Level

Q1. How would you manage a patient expressing immediate suicidal intent?

Answer: I would treat this as an immediate safety emergency, remain with the patient and alert the responsible mental-health team. I would initiate appropriate observation and environmental precautions, support urgent risk assessment and follow the hospital's safety protocol.

Q2. How would you manage a patient experiencing acute psychosis?

Answer: I would assess immediate risk, establish a low-stimulation environment and use non-confrontational communication. I would coordinate urgent psychiatric evaluation, prescribed treatment and continuous reassessment while respecting patient dignity and rights.

Q3. How would you manage a psychiatric patient who becomes violent?

Answer: I would activate the safety response, protect patients and staff, maintain a safe distance and use trained de-escalation methods. Any restrictive intervention must follow applicable law, hospital policy and the least-restrictive approach, with close monitoring and documentation.

Q4. How would you manage severe depression with refusal of food and personal care?

Answer: I would assess nutritional, hydration and self-care risks, evaluate suicide risk and inform the treating team. I would develop a supportive care plan, encourage gradual participation and coordinate psychiatric, medical and nutritional interventions.

Q5. How would you supervise a psychiatric nursing unit?

Answer: I would ensure safe staffing, risk assessment, medication monitoring, therapeutic communication and patient-rights compliance. I would review incidents, provide staff training and coordinate with psychiatrists, psychologists and social workers.

19. ENT Nurse

Fresher Level

Q1. What does ENT stand for?

Answer: ENT stands for Ear, Nose and Throat. It is also known as otolaryngology, a specialty concerned with disorders of these structures and related areas of the head and neck.

Q2. What are common ENT conditions?

Answer: Common conditions include ear infections, sinusitis, tonsillitis, hearing loss, nasal obstruction and throat infections. Some patients may also require surgical treatment.

Q3. What is a tracheostomy?

Answer: A tracheostomy is a surgically created opening in the neck into the trachea to provide an airway. It may be temporary or permanent depending on the patient's condition.

Q4. What is the purpose of an otoscope?

Answer: An otoscope is used to examine the external ear canal and tympanic membrane. It helps identify abnormalities such as inflammation, infection or earwax blockage.

Q5. What is tonsillitis?

Answer: Tonsillitis is inflammation of the tonsils, commonly due to viral or bacterial infection. Symptoms may include sore throat, fever, difficulty swallowing and enlarged tonsils.

Junior Level

Q1. What nursing care is required after tonsillectomy?

Answer: I monitor airway, breathing, vital signs, pain and signs of postoperative bleeding. I follow prescribed diet and fluid instructions and report frequent swallowing, fresh bleeding or respiratory difficulties immediately.

Q2. How do you care for a patient with a tracheostomy?

Answer: I monitor airway patency, breathing, oxygenation and secretion characteristics. I provide tracheostomy care according to protocol, maintain appropriate humidification and ensure emergency airway equipment is available.

Q3. What would you do if a patient develops severe nosebleeding?

Answer: I would ask the patient to sit upright and lean forward, then apply continuous pressure to the soft part of the nose. I would monitor ongoing bleeding and seek urgent medical assistance if bleeding is severe or persistent.

Q4. What are warning signs of an ear infection?

Answer: Warning signs include ear pain, fever, discharge, hearing changes and irritability in children. Severe pain, swelling behind the ear or neurological symptoms require urgent medical evaluation.

Q5. How do you assist a patient undergoing an ENT examination?

Answer: I explain the examination, verify patient identity, prepare required instruments and maintain infection-control precautions. I assist the doctor and document relevant nursing observations.

Senior Level

Q1. How would you manage suspected tracheostomy obstruction?

Answer: I would activate emergency assistance, assess breathing and oxygenation and follow the tracheostomy emergency algorithm. I would provide oxygenation and assist with tube assessment or emergency replacement only within competency and protocol, while preparing alternative airway support.

Q2. How would you manage postoperative bleeding after tonsillectomy?

Answer: I would recognize bleeding as an emergency, activate the ENT response and assess airway, breathing and circulation. I would monitor vital signs, avoid unnecessary oral intake and prepare for urgent medical or surgical intervention.

Q3. How would you supervise tracheostomy care across shifts?

Answer: I would ensure standardized assessment, suctioning competency, emergency equipment readiness and clear documentation. I would verify that staff understand the emergency pathway and can identify obstruction or accidental decannulation.

Q4. How would you manage a suspected deep-neck infection with airway compromise?

Answer: I would immediately escalate the emergency, assess airway and breathing and prepare for specialist airway management. I would ensure appropriate monitoring, IV access and prescribed treatment without unnecessary delay.

Q5. How would you improve infection control in an ENT procedure unit?

Answer: I would establish appropriate instrument reprocessing, hand hygiene, environmental cleaning and safe handling of reusable equipment. I would audit compliance, review infection incidents and provide regular staff training.

20. Plastic Surgery Nurse

Fresher Level

Q1. What is plastic surgery nursing?

Answer: Plastic surgery nursing involves caring for patients undergoing reconstructive, cosmetic and corrective procedures. It includes preparation, wound care, postoperative monitoring and patient education.

Q2. What is the difference between cosmetic and reconstructive surgery?

Answer: Cosmetic surgery primarily aims to improve appearance, whereas reconstructive surgery aims to restore function and appearance after injury, disease or congenital abnormalities.

Q3. What is a skin graft?

Answer: A skin graft is a surgical procedure in which skin is transferred from one body area to another to cover a wound or damaged area. The donor site and graft site both require appropriate postoperative care.

Q4. What is wound dressing?

Answer: Wound dressing covers and protects a wound using suitable materials. It helps maintain an appropriate healing environment, manage drainage and protect against contamination.

Q5. What are common signs of wound infection?

Answer: Common signs include redness, swelling, warmth, increased pain, pus discharge and fever. These findings should be assessed and reported promptly.

Junior Level

Q1. How do you monitor a patient after reconstructive surgery?

Answer: I monitor vital signs, pain, bleeding, wound condition and circulation to the operated area. I also assess swelling, colour changes and signs of infection or graft compromise.

Q2. What would you monitor after free-flap reconstruction?

Answer: I monitor flap colour, temperature, capillary refill, swelling and other prescribed indicators of tissue perfusion. Any sudden change may indicate vascular compromise and requires immediate surgical review.

Q3. How do you care for a skin-graft donor site?

Answer: I maintain the prescribed dressing, monitor drainage and bleeding, assess for infection and support pain management. I educate the patient about dressing care and activity restrictions.

Q4. What would you do if a surgical wound begins to separate?

Answer: I would assess the wound, cover it with an appropriate sterile dressing and notify the surgical team. If evisceration occurs, I would cover exposed organs with sterile saline-moistened dressings, avoid pushing them back and activate emergency assistance.

Q5. How do you educate a patient after cosmetic surgery?

Answer: I explain wound care, medication instructions, activity restrictions, follow-up appointments and warning signs of complications. I encourage the patient to contact the surgical team if unexpected symptoms occur.

Senior Level

Q1. How would you manage suspected vascular compromise in a free flap?

Answer: I would immediately alert the reconstructive surgeon, perform the prescribed flap assessment and compare findings with previous observations. I would avoid delaying surgical evaluation and support urgent intervention while maintaining accurate documentation.

Q2. How would you manage extensive burns requiring reconstructive care?

Answer: I would coordinate wound assessment, infection prevention, pain management, fluid monitoring and nutritional support with the multidisciplinary team. I would also support rehabilitation, psychological care and staged reconstructive planning.

Q3. How would you manage postoperative wound dehiscence?

Answer: I would assess the extent of separation, monitor vital signs and cover the wound using the appropriate sterile dressing. I would urgently notify the surgeon, monitor for infection or evisceration and prepare for further treatment.

Q4. How would you supervise nurses caring for microsurgical reconstruction patients?

Answer: I would verify competency in flap monitoring, wound care, neurovascular assessment and emergency escalation. I would establish standardized observation schedules, review documentation and coordinate with the reconstructive surgical team.

Q5. How would you improve postoperative outcomes in a plastic-surgery unit?

Answer: I would implement standardized monitoring, infection prevention, patient education and early recognition of flap compromise. I would review complications, conduct clinical audits and coordinate rehabilitation and follow-up.

21. Oncology Nurse

Fresher Level

Q1. What is oncology nursing?

Answer: Oncology nursing focuses on patients diagnosed with cancer. It includes treatment support, symptom management, patient education, emotional support and monitoring for complications.

Q2. What is chemotherapy?

Answer: Chemotherapy is cancer treatment using medicines to destroy cancer cells or prevent their growth and division. It may be used alone or combined with surgery, radiation and other treatments.

Q3. What are common side effects of chemotherapy?

Answer: Common side effects include nausea, vomiting, fatigue, hair loss, mouth sores, reduced blood-cell counts and increased infection risk. Effects vary depending on the medicines used.

Q4. What is radiotherapy?

Answer: Radiotherapy uses high-energy radiation to destroy cancer cells or reduce tumour size. It may be used as the main treatment or combined with other cancer therapies.

Q5. What is palliative care?

Answer: Palliative care focuses on improving quality of life by managing pain, symptoms and psychological or social distress. It can be provided alongside cancer-directed treatment and is not limited to end-of-life care.

Junior Level

Q1. What would you do if an oncology patient develops fever during chemotherapy?

Answer: I would recognize possible neutropenic fever, immediately notify the oncology team and assess vital signs and clinical condition. I would assist with urgent investigations and timely prescribed antimicrobial treatment according to the hospital pathway.

Q2. What precautions are required while administering chemotherapy?

Answer: I would verify the treatment protocol, patient identity, drug, dose, route and relevant laboratory results. I would use appropriate hazardous-drug PPE, follow safe administration procedures and monitor for infusion reactions or extravasation.

Q3. What is neutropenia?

Answer: Neutropenia is a reduction in neutrophils, which increases infection risk. Patients may require infection-prevention measures, blood-count monitoring and urgent assessment if fever develops.

Q4. What would you do if chemotherapy extravasation is suspected?

Answer: I would immediately stop the infusion, leave the vascular access device in place initially and follow the drug-specific extravasation protocol. I would notify the oncology team, avoid flushing the line and assist with aspiration or antidote administration when indicated.

Q5. How would you support a patient with severe nausea after chemotherapy?

Answer: I would assess nausea severity, hydration and oral intake, administer prescribed antiemetics and provide appropriate dietary guidance. I would monitor for dehydration and electrolyte disturbances and report persistent symptoms.

Senior Level

Q1. How would you manage suspected neutropenic sepsis?

Answer: I would recognize the emergency, activate the neutropenic-sepsis pathway and immediately assess ABCs and vital signs. I would coordinate urgent cultures and investigations without delaying prescribed broad-spectrum antibiotics, support fluid management and closely monitor response.

Q2. How would you manage suspected vesicant chemotherapy extravasation?

Answer: I would stop the infusion immediately, leave the cannula in place initially, notify the oncology team and follow the drug-specific protocol. I would not flush the line, assist with aspiration and prescribed antidote treatment where indicated, document the area and arrange ongoing assessment.

Q3. How would you manage a patient developing tumour lysis syndrome?

Answer: I would recognize warning signs such as abnormal potassium, phosphate, calcium and uric acid, reduced urine output and cardiac rhythm changes. I would escalate immediately and coordinate prescribed fluid management, laboratory monitoring and treatment with the oncology team.

Q4. How would you supervise nurses administering chemotherapy?

Answer: I would ensure nurses have required competency in hazardous-drug handling, protocol verification, vascular-access assessment and infusion-reaction management. I would conduct audits and ensure compliance with safe administration and waste-disposal procedures.

Q5. How would you manage advanced-cancer pain that remains uncontrolled?

Answer: I would perform a comprehensive pain assessment, identify breakthrough symptoms and communicate urgently with the oncology or palliative-care team. I would support prescribed analgesic adjustments, monitor adverse effects and coordinate physical, psychological and family support.